

Qora

Dynamic Queue & Appointment Management Platform —
Competitive Research, Module Architecture, MVP Roadmap and
Continuous Intelligence System

Primary Market	Secondary Market	Prepared For	Date
Hospitals, Diagnostic Centers, Clinics	Banks, Govt, University, Telecom, Retail	Qora Founding Team (Solo .NET Build)	19 July 2026

Executive Summary

11 RECOMMENDED TOP-LEVEL MODULES	6 COMPETITORS BENCHMARKED	7 BANGLADESH-SPECIFIC WHITE-SPACE FEATURES	3 PHASE ROADMAP: MVP → V1 → V2
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The global category (Qminder, Waitwhile, QLess, WaitWell, Qmatic) has already converged on one truth: appointments, walk-ins, virtual queueing, check-in, notifications and analytics are not separate products — they are one connected patient/customer journey. Vendors that still sell them as disconnected tools (mostly true of every Bangladeshi competitor found in this research) lose on user experience. Qora's core architectural bet should be a **unified Visit Journey** spanning booking → queueing → check-in → service → follow-up, built on a single patient/customer record.

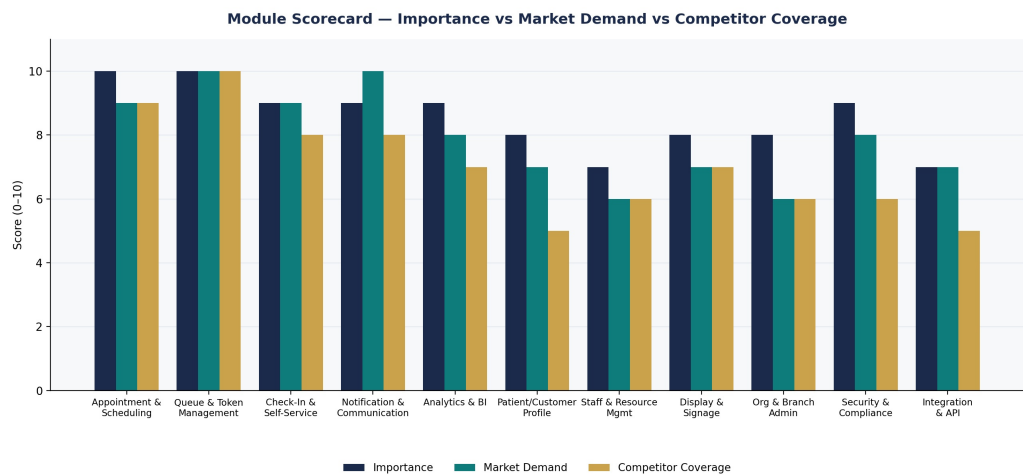
The Bangladesh market is structurally underserved. Every local vendor identified (GCTL, Webprint, Bangla Puzzle, LRL and similar system integrators) sells hardware-centric token/display systems — ticket dispensers, LED boards, on-premise servers — with little to no cloud SaaS delivery, no native Bengali UI, no WhatsApp notifications, no local payment rails (bKash/Nagad), and no offline-tolerant design for unstable connectivity. This is the same gap Qminder filled in the West a decade ago. It is Qora's clearest path to fast adoption.

Global vendors' own customers complain about the same three things repeatedly across G2, Capterra and vendor-comparison sites: (1) hardware lock-in (Qminder's iPad/Apple TV-only kiosk requirement), (2) SMS/messaging costs and reliability, and (3) steep pricing that starts at \$389–\$1,149/month before add-ons — inaccessible for a Bangladeshi clinic or diagnostic center. Qora can win on cloud-native, browser-based hardware independence and a locally-priced subscription.

Recommended posture: Build a healthcare-first MVP around Queue/Token, Appointment, Notification (SMS+WhatsApp) and Display — the four modules with the highest importance and market demand scores — then generalize the same data model outward to banks, government offices, universities and telecom centers, which need near-identical primitives (counters, tokens, priority rules, SLAs) with different vocabulary and compliance rules.

Deliverable 1 — Main Modules & Scorecard

Eleven top-level modules were derived by mapping every capability found across Qminder, Waitwhile, QLess, WaitWell and Qmatic against the specific workflows described for hospitals, diagnostic centers and clinics, then testing each module for reuse across banks, government offices, universities, telecom and retail service centers.



#	Module	Purpose	Imp.	Demand	Coverage
1	Appointment & Scheduling	Pre-booked visits, availability, provider/resource calendars	10	9	9
2	Queue & Token Management	Walk-in + virtual queue, token issuing, routing, priority	10	10	10
3	Check-In & Self-Service	Kiosk / QR / mobile check-in, identity capture	9	9	8
4	Notification & Communication	SMS, WhatsApp, voice call, push, two-way messaging	9	10	8
5	Analytics & Business Intelligence	Wait-time, throughput, staff load, SLA reporting	9	8	7
6	Patient / Customer Profile	Lightweight record, visit history, documents, consent	8	7	5
7	Staff & Resource Management	Counters, doctors/agents, shifts, service-line assignment	7	6	6
8	Display & Digital Signage	TV/LED token boards, waiting-room content, audio call-out	8	7	7
9	Organization & Branch Admin	Multi-branch, service catalog, business rules, roles	8	6	6
10	Security & Compliance	RBAC, audit log, data privacy, HIPAA-equivalent controls	9	8	6
11	Integration & API	Webhooks, REST API, HIS/EHR/CRM/payment connectors	7	7	5

Scoring: 1–10 scale. Importance = criticality to core Qora value proposition. Market Demand = frequency of explicit customer requests across G2/Capterra/Reddit/vendor comparison pages. Coverage = how completely today’s competitors already serve this need (low coverage = whitespace).

Modules considered and rejected as top-level: "Visitor Management" was merged into Check-In & Self-Service (Envoy-style badge printing is a feature, not a module, for Qora’s target verticals). "Billing/Payments" was demoted from top-level to a submodule inside Integration & API for MVP, and promoted only once bKash/Nagad collection becomes a paid feature in V2. "Feedback/Survey" was folded into Analytics as a submodule rather than a standalone module — no competitor treats it as one, and it doesn’t carry enough independent complexity.

Module → Submodule → Feature Hierarchy (1/2)

1. APPOINTMENT & SCHEDULING MODULE

- └ Booking
 - └ Web / mobile self-booking
 - └ Staff-assisted booking (call center / front desk)
 - └ Multi-service booking in one visit (e.g., consultation + lab)
- └ Availability & Calendar
 - └ Doctor/agent/resource calendar
 - └ Slot duration & buffer rules
 - └ Recurring / block scheduling
- └ Rescheduling & Cancellation
 - └ Self-service reschedule link
 - └ Cancellation with reason capture
- └ No-Show Management
 - └ No-show flagging & history
 - └ AI no-show risk prediction
- └ Reminder Management
 - └ SMS reminder Email reminder
 - └ WhatsApp reminder Automated follow-up
 - └ Custom reminder workflow builder

2. QUEUE & TOKEN MANAGEMENT MODULE

- └ Token Issuing
 - └ Kiosk-printed token QR / mobile token
 - └ SMS-issued token (no app/kiosk needed)
- └ Virtual / Remote Queue
 - └ Join queue remotely, live position
 - └ Estimated wait time
 - └ Geo-fenced "leave home now" alert
- └ Queue Routing & Prioritization
 - └ Department / service-line routing
 - └ Priority rules (emergency, elderly, VIP, pre-paid)
 - └ Load-balanced routing across counters/branches
- └ Multi-Stage Queue Flow
 - └ Single token, multiple service stages (OPD → lab → pharmacy)
 - └ Token forwarding between counters

3. CHECK-IN & SELF-SERVICE MODULE

- └ Kiosk Check-in
 - └ Tablet / kiosk check-in
 - └ Barcode / ID scan check-in
- └ Remote Check-in
 - └ QR code check-in Web-link check-in
 - └ Geofenced auto check-in on arrival
- └ Identity & Intake
 - └ Custom intake forms
 - └ Document/insurance upload at check-in
- └ Group / Family Check-in
 - └ Multiple patients under one guardian token

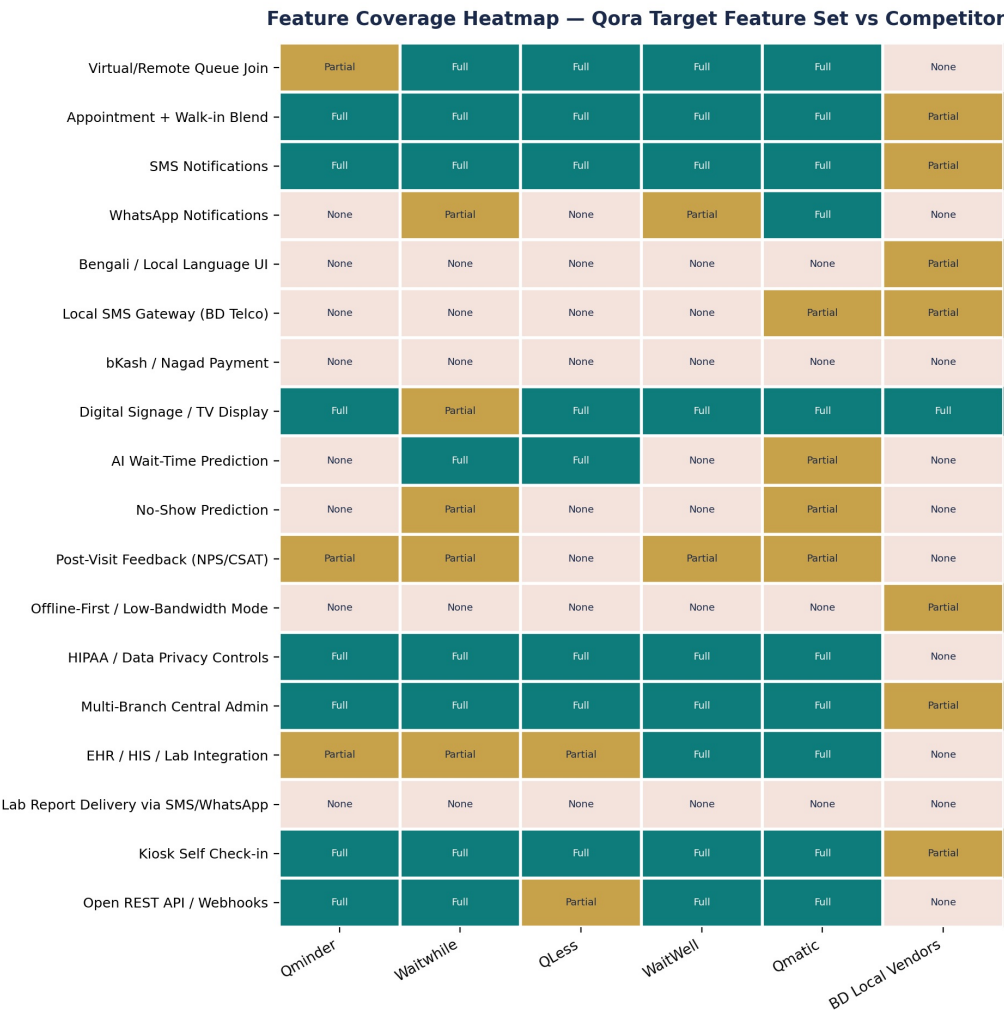
Module → Submodule → Feature Hierarchy (2/2)



Legend: Core (MVP-eligible) · Advanced (V1) · Enterprise (V2)

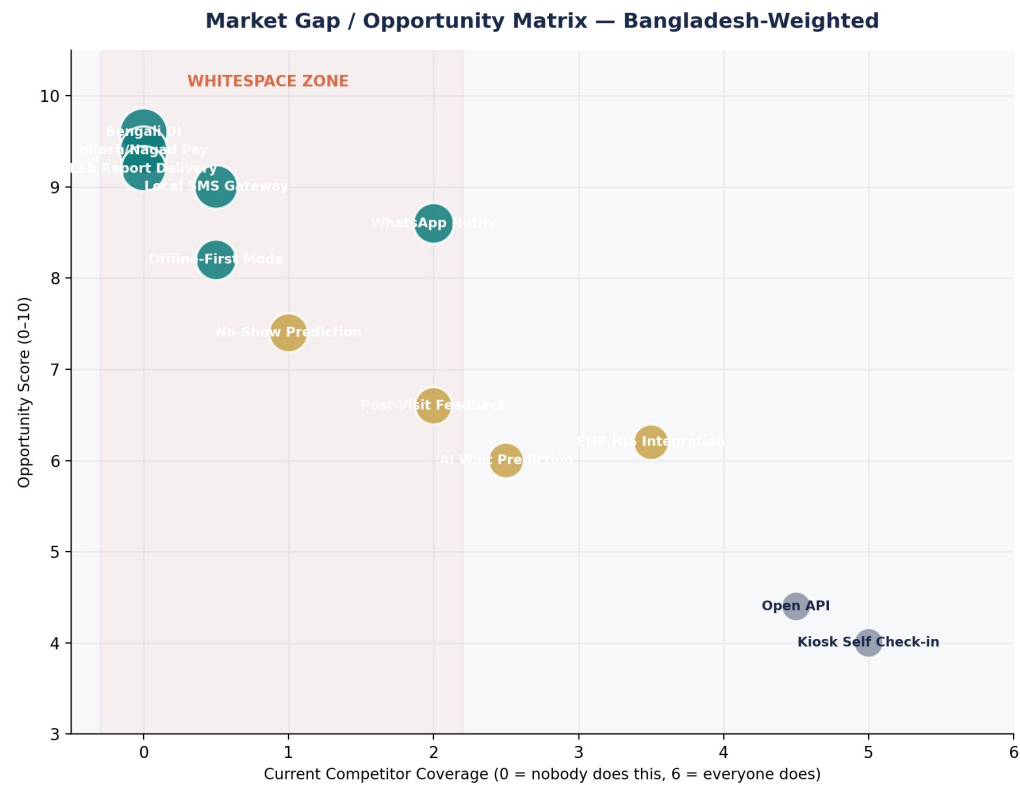
Deliverable 4 — Feature Gap Analysis

The heatmap benchmarks Qora's candidate feature set against five global leaders and the collective Bangladesh local-vendor baseline (GCTL, Webprint, Bangla Puzzle, LRL and similar system integrators, treated as one column since their feature sets are nearly identical: hardware token dispensers + LED display, minimal cloud/SaaS layer).



Market Gap & Opportunity Matrix

Plotting each candidate feature by how little competitors currently offer it (x-axis) against how much it matters to the Bangladesh healthcare buyer specifically (y-axis) surfaces a clear whitespace cluster in the top-left: **Bengali-language UI, bKash/Nagad payment collection, local SMS-gateway reliability, native WhatsApp notifications and SMS/WhatsApp lab-report delivery**. None of the five global leaders localize for Bangladesh, and none of the local hardware vendors offer cloud SaaS, WhatsApp, or digital payment collection.

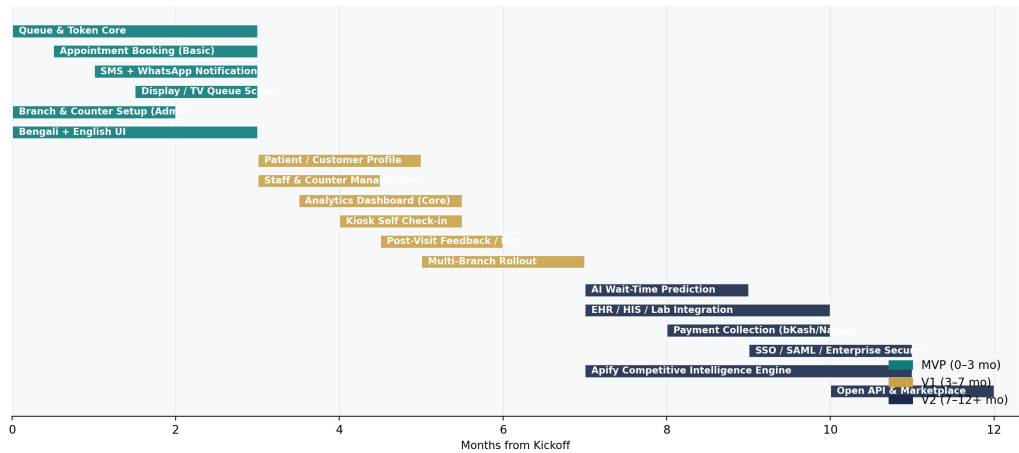


Feature	Competitors Supporting (of 6)	Opportunity Score	Note
Bengali / local-language UI	0	9.6	Zero global vendor localizes; instant differentiator
bKash / Nagad payment collection	0	9.4	No competitor offers local mobile-wallet checkout
Lab report delivery via SMS/WhatsApp	0	9.2	High value for diagnostic centers specifically
Local SMS gateway reliability	0.5	9.0	Global SMS providers are costly/unreliable in BD
Native WhatsApp notifications	2	8.6	Only Qmatic and partially Waitwhile/WaitWell support
Offline-first / low-bandwidth mode	0.5	8.2	No cloud vendor designs for intermittent connectivity
No-show prediction	1	7.4	Rare even among global leaders
Post-visit feedback / NPS	2	6.6	Present but shallow across most vendors
EHR / HIS / Lab-LIS integration	3.5	6.2	Common at enterprise tier only (Qmatic, WaitWell)
AI wait-time prediction	2.5	6.0	Waitwhile leads; differentiator but not urgent for MVP
Kiosk self check-in	5	4.0	Table stakes — must have, but not a differentiator
Open REST API / Webhooks	4.5	4.4	Expected at mid-tier and above; parity feature

Recurring competitor complaints found across G2, Capterra and comparison sites: Qminder users cite iPad/Apple TV hardware lock-in and per-user pricing that becomes expensive at scale (**Pricing**) ; Waitwhile users cite messaging bugs/delays during peak times and a steep automation learning curve (**Reliability**) ; QLess suffered a 2024 bankruptcy/restructuring, creating vendor-stability concerns for government/education buyers (**Stability**) ; all enterprise vendors (Qmatic, QLess, Wavetec) hide pricing behind "contact sales," frustrating small clinics (**Transparency**) .

Deliverable 5 — MVP Recommendation & Roadmap

Qora MVP → V1 → V2 Roadmap (Solo .NET Developer Pace)



MUST BUILD NOW (MVP)

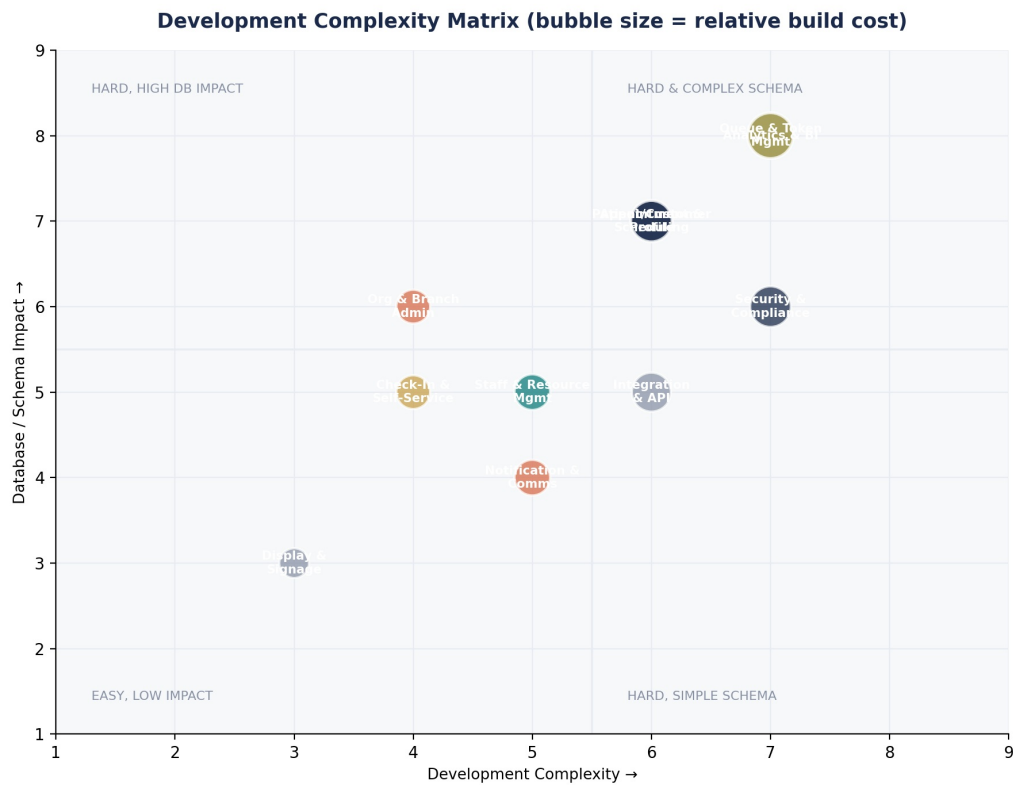
Queue & Token core (issue, route, call, multi-stage)	Patient/customer profile & visit history
Basic appointment booking + calendar	Staff/counter performance analytics
SMS + WhatsApp notifications (single gateway)	Kiosk self check-in, QR check-in
Digital display / TV token board	Feedback/NPS survey
Branch, counter & service-catalog admin	AI wait-time & no-show prediction
Bengali + English UI	EHR/HIS/Lab-LIS integration, bKash/Nagad payments
Basic RBAC (admin / staff / display)	SSO/SAML, enterprise audit & Apify intelligence engine

BUILD LATER (V1 → V2)

IGNORE (FOR NOW)

Native iOS/Android apps (mobile web is sufficient at MVP), video/telehealth consultation, in-house payment processing licensing, hardware manufacturing (dispensers/kiosks) — partner with existing BD hardware vendors instead of competing with them directly.

Deliverable 6 — Database & Development Complexity

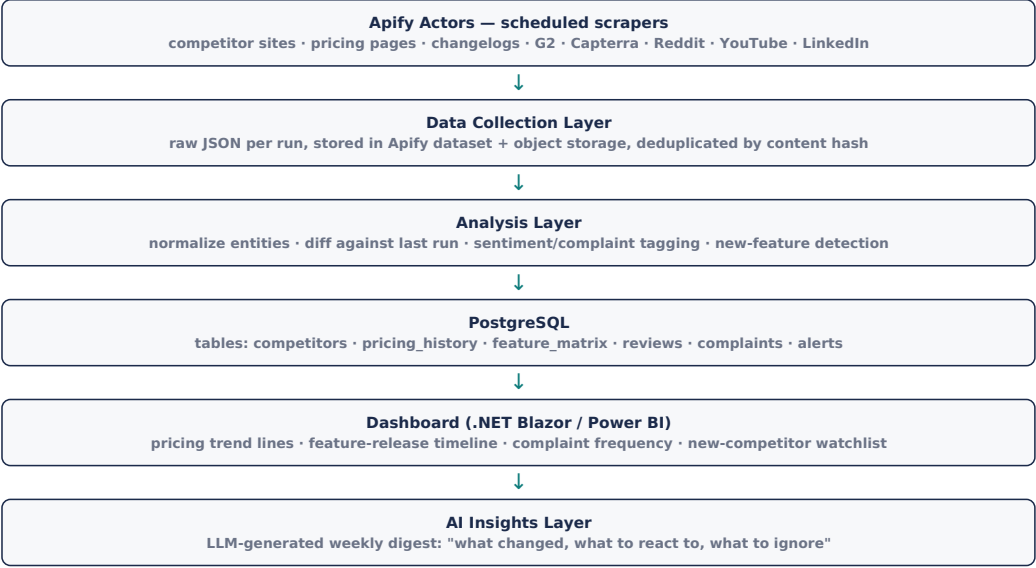


Module	Complexity	DB Impact	Dev Cost	Maintenance
Queue & Token Management	Hard	Hard	Hard	Medium
Analytics & Business Intelligence	Hard	Hard	Hard	Medium
Appointment & Scheduling	Medium	Hard	Medium	Medium
Patient / Customer Profile	Medium	Hard	Medium	Hard (compliance)
Security & Compliance	Hard	Medium	Medium	Hard
Integration & API	Medium	Medium	Medium	Hard (external deps)
Notification & Communication	Medium	Easy	Medium	Medium (gateway costs)
Staff & Resource Management	Medium	Medium	Medium	Easy
Organization & Branch Admin	Easy	Medium	Easy	Easy
Check-In & Self-Service	Easy	Easy	Easy	Easy
Display & Digital Signage	Easy	Easy	Easy	Easy

Queue & Token and Analytics carry the heaviest schema and concurrency load (real-time state, multi-branch consistency, historical aggregation) — build these on a clean event-sourced token-state table from day one to avoid a costly V1 rewrite. Notification is cheap to build but has an ongoing per-message cost that must be priced into subscription tiers.

Deliverable 7 — Apify Continuous Intelligence System

A standing pipeline keeps Qora's competitive picture current after this one-time report, so pricing changes, new features and recurring complaints from Qminder, Waitwhile, QLess, WaitWell, Qmatic and any new Bangladesh entrant surface automatically.



WHAT IT MONITORS

Signal	Cadence	Trigger
Competitor pricing pages	Weekly	Alert on any price/tier change
Changelogs / release notes	Weekly	Alert on new feature launch
G2 / Capterra reviews	Bi-weekly	Flag recurring complaint themes
Reddit / forums	Weekly	Flag unmet feature requests
New competitor discovery	Monthly	Search-based crawl for new BD/regional entrants

Final Answer — The Adoption-Maximizing Architecture

Build Qora around one unified Visit Journey engine, not four separate products bolted together. The data model that wins in a hospital OPD (patient → token → doctor → lab → report) is structurally the same as a bank branch (customer → token → teller → service → receipt) or a government office (citizen → token → counter → service → document). Keep the four MVP modules — Queue & Token, Appointment, Notification, Display — vertical-agnostic at the schema level, and push all healthcare-specific language ("patient", "OPD", "doctor") into a configurable terminology/label layer rather than hard-coded entities. That single decision is what lets the same MVP expand into banks, universities, telecom and government without a rebuild.

Win healthcare first by shipping the two things no Bangladeshi competitor has and every global leader charges \$400–\$1,000+/month for: cloud-native queueing with WhatsApp/SMS notifications in Bengali, and browser-based displays that don't require an iPad/Apple TV kiosk. Price it for a mid-size diagnostic center, not a US hospital network, and the whitespace identified in the Opportunity Matrix — Bengali UI, bKash/Nagad, local SMS reliability, WhatsApp report delivery — becomes the wedge that gets Qora into its first ten clinics before the global vendors even notice the market.

This report was compiled from publicly available vendor pricing pages, product pages, G2, Capterra, GetApp, SoftwareSuggest and Bangladesh market listings current as of July 2026. Enterprise pricing for Qmatic, QLess and Wavetec is quote-based and not published; figures cited are directional.